

Culturally Tailored Diabetes Prevention Interventions Among Filipino American Adults

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Introduction

Type 2 diabetes mellitus is a major public health concern in the United States, affecting more than 30 million adults and contributing substantially to morbidity, mortality, and healthcare costs.

Diabetes increases the risk of cardiovascular disease, kidney failure, neuropathy, vision loss, and premature death, making prevention and early intervention critical public health priorities.

Healthy People 2020 identify diabetes reduction as a national objective, emphasizing lifestyle modification, early detection, and culturally appropriate interventions to reduce disease burden.

Filipino Americans represent one of the fastest-growing Asian American subgroups in the United States and experience disproportionately high rates of type 2 diabetes compared to non-Hispanic White populations. Research indicates that Filipino Americans often develop diabetes at lower body mass index (BMI) levels, which complicates risk identification and prevention efforts.

Cultural dietary patterns, genetic predisposition, limited access to culturally competent care, and underutilization of preventive services contribute to elevated risk. Given these disparities, diabetes prevention among Filipino American adults is an important and justified focus for health promotion efforts aligned with Healthy People 2020 objectives.

Literature Review

A review of the literature identified five evidence-based interventions that have demonstrated effectiveness in diabetes prevention and management, particularly among minority and immigrant populations.

Community-Based Diabetes Prevention Programs

Community-based diabetes prevention programs (DPPs) adapted from the original Diabetes Prevention Program have been widely implemented in diverse populations. These interventions

emphasize lifestyle modification through structured education, goal setting, and peer support. Studies demonstrate significant reductions in fasting glucose levels, weight loss, and improved physical activity among participants. Community settings such as cultural centers and nonprofit organizations increase accessibility and engagement, particularly for ethnic minority groups (Islam et al., 2013).

Culturally Tailored Nutrition Education

Nutrition education interventions tailored to cultural food practices have shown improved dietary behaviors and glycemic outcomes among Asian American populations. For Filipino Americans, culturally adapted programs address traditional foods such as rice, fried dishes, and sugary beverages while promoting healthier preparation methods. Outcomes include improved dietary knowledge, reduced carbohydrate intake, and improved hemoglobin A1c (HbA1c) levels (Utz et al., 2014).

Physical Activity Promotion Programs

Physical activity interventions that incorporate culturally acceptable forms of exercise, such as group walking programs or dance-based activities, have demonstrated success in improving insulin sensitivity and reducing diabetes risk. These programs often emphasize social support and community engagement, which are particularly effective in collectivist cultures. Participants experience improvements in physical fitness, weight management, and self-efficacy for physical activity (Murrock & Higgins, 2009).

Family-Centered Lifestyle Interventions

Family-centered interventions recognize the influence of family dynamics on health behaviors. Programs that involve family members in education sessions and lifestyle changes have been

shown to improve adherence and long-term behavior change. Among Filipino Americans, strong family ties make this approach especially relevant. Outcomes include improved dietary habits, increased physical activity, and enhanced social support for behavior change (Baig et al., 2015).

Faith- and Community Organization–Based Interventions

Faith-based interventions delivered through churches and religious organizations have been effective in promoting diabetes awareness and prevention among Filipino Americans. These interventions leverage trust, cultural relevance, and existing social networks. Studies report increased screening rates, improved diabetes knowledge, and positive behavior changes related to diet and physical activity (Trinh-Shevrin et al., 2015).

Proposed Health Promotion Intervention

Based on findings from the literature, a culturally tailored diabetes prevention program for Filipino American adults is proposed. The program, titled “**Kalusugan sa Komunidad**” (Health in the Community), is a community-based lifestyle intervention aimed at reducing diabetes risk through education, physical activity, and social support.

Program Description and Activities

The intervention will target Filipino American adults aged 18 and older and will be delivered through community centers and faith-based organizations. The program will consist of weekly sessions over a 12-week period and will include the following components:

1. Diabetes Education Workshops

Educational sessions will address diabetes risk factors, prevention strategies, and the

importance of early detection. Materials will be culturally tailored and available in English and Tagalog.

2. Culturally Adapted Nutrition Education

Participants will receive guidance on modifying traditional Filipino meals to reduce sugar and refined carbohydrate intake while preserving cultural relevance. Activities will include cooking demonstrations, menu planning, and label-reading exercises.

3. Physical Activity Promotion

Group-based physical activities such as walking clubs and Filipino dance sessions will be incorporated to promote regular physical activity in an enjoyable and culturally familiar manner.

4. Family and Social Support Integration

Family members will be encouraged to attend selected sessions to foster a supportive home environment and reinforce lifestyle changes.

5. Health Screenings and Referrals

Participants will receive baseline and follow-up screenings for BMI, blood pressure, and glucose levels, along with referrals to primary care providers when necessary.

Theoretical Framework

The proposed intervention is grounded in the **Health Belief Model (HBM)** and **Social Cognitive Theory (SCT)**. HBM addresses perceive susceptibility to diabetes, perceived severity of complications, perceived benefits of lifestyle changes, and barriers to behavior change.

Educational components are designed to increase risk awareness and motivate preventive action.

Social Cognitive Theory supports the inclusion of group-based activities and peer modeling to enhance self-efficacy and observational learning. Social support and reinforcement are integrated through group sessions and family involvement, facilitating sustained behavior change.

Evaluation Plan

A comprehensive evaluation plan will be used to assess the effectiveness of the intervention.

Evaluation Design

A pretest–posttest design will be utilized to measure changes in knowledge, behaviors, and biometric outcomes.

Process Evaluation

- Attendance and participation rates
- Participant satisfaction surveys
- Fidelity of program implementation

Impact Evaluation

- Changes in diabetes knowledge
- Self-reported dietary and physical activity behaviors
- Self-efficacy for lifestyle modification

Outcome Evaluation

- Changes in BMI and waist circumference
- Blood glucose and HbA1c levels

- Blood pressure measurements

Data will be collected at baseline, immediately post-intervention, and three months following program completion.

Barriers and Limitations

Several barriers may affect the success of the proposed intervention. Cultural attachment to traditional foods, time constraints due to work and family responsibilities, and limited access to healthcare services may reduce participation and adherence. Language barriers and varying levels of health literacy may also limit comprehension of health information.

Limitations include reliance on self-reported behavior data, which may be subject to bias, and challenges in sustaining long-term behavior change beyond the intervention period. Additionally, resource limitations may impact program scalability and long-term sustainability.

Conclusion

Type 2 diabetes remains a significant public health issue among Filipino American adults.

Culturally tailored interventions that incorporate education, physical activity, family involvement, and community engagement offer promising strategies for diabetes prevention. The proposed **Kalusugan sa Komunidad** program aligns with Healthy People 2020 objectives and addresses key cultural and social factors influencing health behaviors. By reducing diabetes risk and promoting healthier lifestyles, this intervention has the potential to improve health outcomes and reduce disparities within the Filipino American community.

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